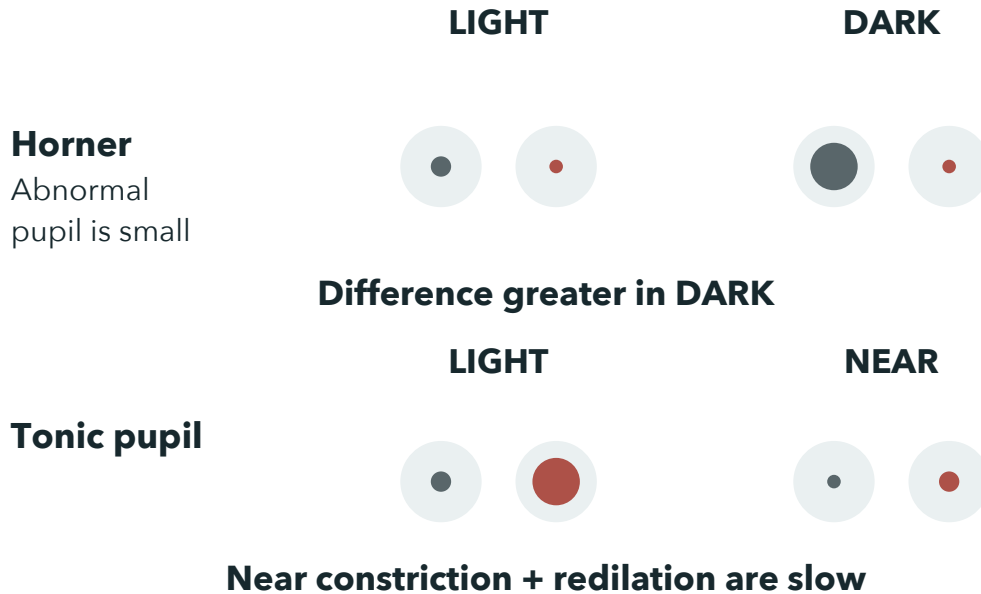


Ptosis, pupil patterns and fatigability

CLINICAL TYPE B | Focused decisions for M3-M5

The pupil changes the localisation

Compare light, dark and near responses



Grey: comparison pupil Red: affected pupil

Original pupil schematic. The abnormal pupil is coloured; examples are qualitative.

Associated signs narrow the pathway

Finding	Clinical inference
Small pupil + mild ptosis; worse anisocoria in dark	Sympathetic dysfunction. Shoulder/medial-arm symptoms suggest a second-order lesion near the pulmonary apex.
Large pupil; weak light, tonic near response	Postganglionic parasympathetic dysfunction is possible when movements/lids are otherwise normal.
Large pupil + III motor weakness	Compression remains an emergency concern; the motor examination must accompany pupil testing.

References AAO: Horner syndrome | AAO: Adie tonic pupil

Fatigability requires systemic questions

Suspected ocular myasthenia

- 1 At first assessment, ask about swallowing, speech, chewing, orthopnoea and breathlessness. New bulbar/respiratory weakness → urgent hospital assessment.
- 2 If clinically stable, variable ptosis/diplopia with normal pupils → assess fatigability and the broader differential.
- 3 Negative receptor antibodies do not exclude disease; specialist electrophysiology may clarify seronegative cases.

Do not rely on one reassuring number

Finding	Clinical inference
Normal oxygen saturation	Does not exclude dangerous myasthenic respiratory weakness. Assess respiratory strength and airway safety.
Incomplete lid closure	Protect the corneal surface while investigating the cause: lubrication and appropriate closure support.

Keep ptosis and exposure separate

A drooping lid does not guarantee safe closure. Record blink, lagophthalmos and corneal staining, especially with facial weakness.